

Patient Name: Sarah Jenkins **Date of Birth:** 04/21/1953 (Age: 72) **Medical Record Number:** SJK210453-01 **Date of Encounter:** August 26, 2025

Subjective

Ms. Jenkins is a 72-year-old female presenting for her annual wellness visit. She reports her overall health is fair but notes increasing shortness of breath with exertion, even while walking around her home. She experiences some mild, non-radiating chest pressure intermittently. She denies orthopnea, but states she uses two pillows to sleep at night. She has been monitoring her blood sugar levels at home, with readings typically in the 150-180 mg/dL range. She reports fatigue and has noticed mild swelling in her ankles by the end of the day. She has a history of smoking for 30 years, quitting 10 years ago.

Objective

Vitals:

- **BP:** 148/88 mmHg
- **HR:** 82 bpm, regular
- **Temp:** 98.2°F (36.8°C)
- **O2 Sat:** 94% on room air
- **Weight:** 185 lbs (83.9 kg)
- **BMI:** 31.7 kg/m²

Physical Exam:

- **General:** Well-developed, slightly overweight female in no acute distress.
- **Cardiovascular:** Regular rate and rhythm. S1, S2 audible. No murmurs, rubs, or gallops. Trace pitting edema noted in bilateral ankles.
- **Pulmonary:** Diminished breath sounds at the bases bilaterally. Scattered crackles on auscultation, which clear with cough. No wheezing or rhonchi.
- **Extremities:** Palpable dorsalis pedis and posterior tibial pulses. No signs of infection or ulceration.

Laboratory & Diagnostic Results:

- **Glucose:** 175 mg/dL
- **HbA1c:** 8.5%
- **Creatinine:** 2.1 mg/dL
- **eGFR:** 27 mL/min/1.73m²
- **BNP:** 1120 pg/mL
- **Chest X-ray (CXR):** Cardiomegaly and mild pulmonary vascular congestion.

Assessment and Plan

Assessment: Patient is a 72-year-old female with a complex and chronic medical history, with the following conditions documented, evaluated, and treated during this encounter:

1. **Congestive Heart Failure (CHF), unspecified: (HCC 85)**
 - *ICD-10 Code: I50.9*
 - Ms. Jenkins presents with classic symptoms of heart failure, including dyspnea on exertion, fatigue, and lower extremity edema. The elevated BNP level and radiographic evidence of cardiomegaly and pulmonary congestion support this diagnosis. Her medication regimen (Furosemide) has been adjusted.
2. **Type 2 Diabetes Mellitus with Chronic Kidney Disease: (HCC 18)**
 - *ICD-10 Code: E11.22*
 - Patient's uncontrolled diabetes (HbA1c of 8.5%) and elevated creatinine/low eGFR indicate a clear linkage between these two conditions. This diagnosis reflects the complex interaction and resource utilization associated with managing both diseases concurrently.
3. **Chronic Kidney Disease, Stage 4 (Moderate to Severe): (HCC 134)**
 - *ICD-10 Code: N18.4*
 - As a direct consequence of her long-standing diabetes, Ms. Jenkins has progressive renal impairment. Her eGFR of 27 mL/min/1.73m² places her in Stage 4. This is a severe complication that significantly increases her risk and resource needs.
4. **Chronic Obstructive Pulmonary Disease (COPD), unspecified: (HCC 111)**
 - *ICD-10 Code: J44.9*
 - Given her significant smoking history and physical exam findings of diminished breath sounds, this diagnosis is being maintained and managed. Her respiratory symptoms are likely compounded by her CHF, but the underlying COPD is a chronic, separately managed condition.
5. **Morbid Obesity due to excess calories (BMI 31.7): (HCC 22)**
 - *ICD-10 Code: E66.01, Z68.41*
 - Ms. Jenkins' BMI is 31.7. This condition contributes to her overall health risk, exacerbates her other chronic illnesses, and requires ongoing management and counseling.

Plan:

- **Heart Failure:** Increase Furosemide to 40 mg daily. Continue Lisinopril and Metoprolol. Follow up with Cardiology in 2 weeks.
- **Diabetes/CKD:** Increase Metformin dose and add SGLT2 inhibitor. Counsel on diet and exercise. Referral to Nephrology for co-management.
- **COPD:** Continue home oxygen use as needed. Encourage pulmonary rehabilitation program.
- **Obesity:** Referral to a nutritionist for diet and weight loss counseling.
- **Patient Education:** Provided extensive counseling on the interrelation of her conditions. Emphasized the importance of medication adherence, diet modification, and smoking cessation.

This documentation captures the patient's full complexity and supports the appropriate HCC coding, demonstrating the severity of her condition and the resources required for her care.